

MCHB FY27 Title V Block Grant Guidance

Core Guidance Requirements by MCH Population Health Domain

OVERVIEW

Title V Block Grant programs must address five MCH population domains plus one optional cross-cutting domain. This guide details the core requirements, available National Performance Measures (NPMs), and key application/reporting areas for each domain.

DOMAIN 1: WOMEN/MATERNAL HEALTH

Legislative Foundation:

Preventive and primary care services for all pregnant women, mothers, and infants up to age one

Purpose:

To provide access to quality maternal health services, reduce maternal mortality and morbidity, improve postpartum care, and support family planning and reproductive health across the life course.

Universal NPM – REQUIRED

Postpartum Visit (2-part measure)

A) Percent of women who attended a postpartum checkup within 12 weeks after giving birth

B) Percent of women who attended a postpartum checkup and received recommended care components

Measure Domain: Clinical Health Systems

Focus: Addresses maternal mortality/morbidity by ensuring comprehensive postpartum clinical services including screening, counseling, and management of health issues

Key Application/Reporting Areas

Program Structure & Clinical Services:

- State systems and partnerships supporting prenatal care delivery
- Maternal specialty provider availability and access
- Integration of behavioral/mental health services into prenatal care
- Management of high-risk pregnancies

Health Behaviors & Prevention:

- Family planning counseling and access to contraception
- Substance use screening and support during pregnancy
- Prenatal nutrition, exercise, and wellness education

Community Health Factors:

- Addressing racial/ethnic disparities in maternal outcomes
- Medicaid enrollment and EPSDT services for pregnant women
- Coordination with WIC and other social services
- Housing and food security support

DOMAIN 2: PERINATAL/INFANT HEALTH

Legislative Foundation:

Preventive and primary care services for pregnant women, mothers, and infants up to age one

Purpose:

To reduce stillbirth, infant mortality, and preventable diseases; improve newborn screening and early intervention; promote safe infant sleep and optimal infant health practices.

Universal NPM – REQUIRED

Postpartum Visit (see Women/Maternal Health domain)

Applies here as well; counts once toward minimum 5 NPMs across domains

Key Application/Reporting Areas

Clinical Systems & Perinatal Care:

- Hospital birthing capacity and NICU/Level of Care distribution
- Perinatal regionalization policies and protocols
- Obstetric provider availability (OB/GYN, midwives, maternal-fetal medicine)

Health Behaviors & Infant Care:

- Breastfeeding promotion and lactation support
- Safe sleep education and infant sleep positioning practices
- Newborn care education

Community Health Factors & Social Determinants:

- Addressing racial/ethnic disparities in infant mortality
- Housing, food, and economic security for pregnant women and new mothers
- Tribal consultation and services for American Indian/Alaska Native populations

DOMAIN 3: CHILD HEALTH

Legislative Foundation:

Preventive and primary care services for children (ages 1–21)

Purpose:

To promote health and well-being across childhood; prevent and manage chronic conditions; ensure access to preventive care, immunizations, oral health, behavioral/mental health, and injury prevention.

Universal NPM – REQUIRED

Medical Home – Overall & Sub-components

Percent of children with and without special health care needs, ages 0 through 17, who have a medical home

Measure Domain: Clinical Health Systems

Key Application/Reporting Areas

Clinical Health Systems:

- Primary care provider availability and access for children
- Medical home/primary care system development
- Developmental screening integration in clinical settings
- Immunization programs and provider training
- Oral health provider network and dental home establishment

Health Behaviors & Prevention:

- Nutrition and healthy eating promotion
- Physical activity and exercise programs
- Injury prevention (vehicle, water, home, sports safety)

Community Health Factors & Social Determinants:

- Food security and WIC coordination
- Housing stability and homelessness prevention
- Transportation access
- Addressing racial/ethnic health disparities

DOMAIN 4: ADOLESCENT HEALTH

Legislative Foundation:

Preventive and primary care services for children (ages 12–21)

Purpose:

To address unique adolescent health needs; promote mental health and substance use prevention; support healthy development, sexual health, and injury prevention.

Key Application/Reporting Areas

Clinical Health Systems:

- Access to adolescent-appropriate primary and preventive care
- Mental health and behavioral health provider capacity
- Substance use screening and treatment services
- Sexual and reproductive health services

Health Behaviors & Prevention:

- Tobacco, vaping, and smoking prevention
- Substance use prevention and treatment
- Violence prevention and intervention
- Injury prevention (vehicle safety, suicide prevention, sports injuries)

Community Health Factors & Social Determinants:

- Mental health support and peer mentorship programs
- Addressing bullying and cyberbullying
- LGBTQ+ youth support and affirming services
- Homelessness and housing instability prevention

DOMAIN 5: CHILDREN WITH SPECIAL HEALTH CARE NEEDS (CSHCN)

Legislative Foundation:

Family-centered, community-based, coordinated care for children with special health care needs, including care coordination services

Purpose:

To ensure CSHCN receive comprehensive, coordinated, family-centered care; promote well-functioning systems of care; support transition to adult health care.

Universal NPM – REQUIRED

Medical Home – Overall (see Child Health domain)

REQUIRED in both Child Health and CSHCN domains; counts as 1 NPM toward minimum of 5

Key Application/Reporting Areas

CSHCN System of Services Components:

- Care coordination and case management programs
- Medical home development and primary care access
- Specialty care provider networks and referral systems
- Rehabilitation services (physical, occupational, speech therapy)
- Transition planning from pediatric to adult health care

Family Partnership & Engagement:

- Family advisory committees and decision-making roles
- Family-to-Family Health Information Centers (F2F) support
- Peer support and family mentorship programs

- Family leadership training and stipends

Health Insurance & Financial Support:

- Medicaid and CHIP eligibility, enrollment, and coverage optimization
- Waiver programs for home and community-based services
- Coordination between Title V and Medicaid regarding CSHCN services

DOMAIN 6: CROSS-CUTTING & SYSTEMS BUILDING

Purpose:

To address priority needs that apply across multiple MCH population domains or relate to program capacity, systems integration, organizational change, and foundational infrastructure supporting all MCH services.

Available Measures:

National Performance Measures (NPMs): None currently specified in the NPM framework

State Performance Measures (SPMs) - OPTIONAL (not required)

States may develop SPMs to address cross-cutting priority needs, such as:

Key Application/Reporting Areas

Family & Community Partnerships:

- Organizational vision and commitment to family partnership
- Contracting with family-led organizations
- Advisory committee structure and family leadership roles

Workforce Development & Capacity:

- Title V program structure and staffing levels
- MCH workforce composition and training needs
- Recruitment and retention initiatives

MCH Data Systems & Capacity:

- Data collection, analysis, and dissemination infrastructure
- Data linkages across systems (health, education, social services)
- Real-time data for rapid response to emerging issues

Equity & Disparity Reduction:

- Health equity framework and integration across all domains
- Disaggregated data and intersectional analysis
- Strategies to address racial/ethnic, geographic, and socioeconomic disparities
- Culturally tailored program adaptations

KEY THEMES IN FY27 GUIDANCE (2025 UPDATE)

1. Community Health Factors as a Measure Domain

Enhanced focus on social determinants: housing, food security, transportation, childcare, environmental health. Discrimination and health equity as explicit NPM options.

2. Family Partnership as Core Capacity

Formal definition: patients, families, and health professionals working in active partnership at various levels. Required demonstration of family engagement in decision-making and program planning.

3. Life-Course Approach

Recognition of how early experiences shape lifelong health. Continuity of care from preconception through adolescence.

4. Data-Driven Programming & Accountability

Three-tiered framework: NOMs, NPMs, ESMs (plus SPMs). Required measurement of progress on short- and medium-term outcomes.

5. Health Equity & Disparity Reduction

Explicit examination of disparities by race/ethnicity, income, geography. Culturally responsive and trauma-informed services.

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